

1. Administrative & Study Identification

Full Study Title: A National, Prospective, Non-Interventional Study (NIS) of Nivolumab (BMS-936558) With or Without Ipilimumab (BMS-734016) in Patients With Advanced Renal Cell Carcinoma in Real Life Setting
Short Title/Acronym: WITNESS
Protocol Number: CA209-9GY
NCT Number: NCT03455452
Sponsor Name: Bristol-Myers Squibb
Investigational Product: Nivolumab (with or without Ipilimumab)
Phase of Development: Observational / Phase IV (Non-interventional)
Medical Monitor: Bristol-Myers Squibb Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effectiveness, specifically overall survival (OS), of nivolumab (monotherapy or in combination with ipilimumab) in patients with advanced Renal Cell Carcinoma (aRCC) treated in routine clinical practice. **Secondary Objectives:** To evaluate progression-free survival (PFS), duration of treatment (DOT), duration of response (DOR), overall response rate (ORR), disease control rate (DCR), and treatment-related adverse events (TRAEs).

2.2 Background & Rationale Nivolumab is an immune checkpoint inhibitor approved for the treatment of advanced renal cell carcinoma. While pivotal phase III clinical trials (such as CheckMate 025) have demonstrated significant survival benefits, the WITNESS study was initiated to confirm the real-world effectiveness and safety of this therapy in a broader, unselected patient population in routine clinical settings across France.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Prospective, Non-interventional) **Control Type:** Single-arm (Real-world cohorts)
Blinding: Open-label (Routine Practice) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: Approximately 732 patients **Number of Sites:** 62 to 69 centers throughout France,
Estimated Study Start: 21-Jan-2018 **Estimated Primary Completion:** 30-Nov-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult participants (age ≥ 18 years) with a histologically or cytologically confirmed diagnosis of advanced renal cell carcinoma (aRCC). 2. The patient's physician has already decided to initiate treatment with nivolumab (as a 2nd or 3rd line therapy) or nivolumab plus ipilimumab for the first time, in accordance with the label approved in France.

4.2 Exclusion Criteria 1. Primary diagnosis of a cancer other than advanced RCC within the past five years that requires systemic or other treatment. 2. Previous treatment with anti-PD1, anti-PDL1, or anti-CTLA4 therapy. 3. Current participation in an interventional clinical trial for advanced RCC.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Nivolumab administered according to standard routine clinical practice and the French marketing authorization. **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Patients are followed for 3 years (36 months) from the index date (treatment initiation) until disease progression, unacceptable toxicity, loss to follow-up, or death.

5.2 Concomitant & Prohibited Therapy Permitted: Standard-of-care concomitant medications for symptom management and co-morbidities as prescribed by the treating physician in a real-world setting. **Prohibited:** Concurrent participation in an interventional clinical trial.

6. Study Timeline & Visit Schedule

(Note: As an observational study, assessments align with standard routine clinical practice. Data entry in the electronic case report form (eCRF) follows specific milestones).

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Baseline	Day 0 Treatment decision, First Dose of Nivolumab, Baseline eCRF Entry Interim 1 Week 6 Routine care evaluation, Safety/TRAEs tracking, eCRF Data Entry
Interim Follow-ups	Months 3, 6, 9, 12, 18, 24, 30	Routine Oncology Visits, Effectiveness Assessment (Tumor response), Safety recording, eCRF Data Entry End of Study Month 36 Final survival tracking, Exit eCRF Entry	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS), defined as the time from the first administration of nivolumab to death

from any cause. **Measurement Method:** Tracked via medical records and routine clinical survival follow-ups over a 3-year period.

7.2 Secondary & Exploratory Endpoints * **Progression-Free Survival (PFS):** Defined as the time from the first administration of nivolumab to the first disease progression or death due to any cause. * **Overall Response Rate (ORR) and Duration of Response (DOR):** Investigator-assessed proportion of patients with complete or partial response, and the duration of that response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Collection of treatment-related adverse events (TRAEs) of any grade via standard clinical evaluation. **Serious Adverse Events (SAEs):** Monitoring of Grade 3 and serious TRAEs, as well as AEs leading to treatment discontinuation. **Data Monitoring Committee (DMC):** Supervised by Bristol-Myers Squibb's pharmacovigilance and observational research teams.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: The Safety Analysis Set includes all patients who receive at least one nivolumab infusion. The Effectiveness Analysis Set includes a subpopulation of the safety set that meets all inclusion criteria. **Sample Size Justification:** As an observational, real-world study, the sample is determined by routine practice uptake (estimated at over 300 patients per major cohort) to provide robust descriptive statistics and confidence intervals. **Handling of Missing Data:** Subgroup analyses are conducted in a complete case manner using descriptive statistics. Kaplan-Meier methods are used to estimate OS and PFS, with censoring for patients lost to follow-up or enrolling in interventional trials.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: The study is conducted in accordance with Good Pharmacoepidemiology Practices (GPP), the Declaration of Helsinki, and applicable French regulations for non-interventional studies. **Monitoring Plan:** Periodic central data review and source data verification from electronic medical records into the eCRFs. **Audit Trail:** eCRF locking after predefined milestones (e.g., interim data locks) and routine data clarification procedures.

11. Study Identifiers & Governance

Primary ID: CA209-9GY **Registry Number:** NCT03455452 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** WITNESS **Official Regulatory Title:** A National, Prospective, Non-Interventional Study (NIS) of Nivolumab (BMS-936558) With or Without Ipilimumab (BMS-734016) in Patients With Advanced Renal Cell Carcinoma in Real Life Setting

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced Renal Cell Carcinoma (aRCC) **Diagnosis Threshold:** Histologically or cytologically confirmed aRCC **Medical Methodology:** Routine Clinical Practice / Real-World Evidence **Optimization Target:** Overall Survival (OS) and Progression-Free Survival (PFS)

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for aRCC in France **Education Protocol (HCP):** Site initiation and training for eCRF data entry per observational protocol **Education Protocol (Patient):** Standard informed consent for real-world health data collection and privacy **Quality Audit Mechanism:** Source data verification against routine electronic medical records (EMR)

14. Observation & Follow-Up Schedule

Total Duration: 36 months (3 years) **Onsite Visit Cadence:** Aligned with routine care (Data entered at Day 0, Week 6, and Months 3, 6, 9, 12, 18, 24, 30, and 36) **Remote Monitoring Frequency:** Periodic central data review **Checklist Review Interval:** Routine oncology follow-ups

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				